جوستاف روسي انترناشيونال - مصر (EPE) *

طلب علاج أول مرة / متردد

إسم المريض رباعياً : شيماء حسن سيد فرج
المحافظة : القاهره **الحي / المدينة :** عين شمس **العنوان :** 0
رقم البطاقة : 28708091402708 **رقم التليفون :** 01254845265 **تاريخ تسجيل الإستمارة :** 24-10-2026
العمر(تاريخ الميلاد) : 09-08-1987 **المهنة :** ربه منزل

الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل:-التأمين الصحي-القوات المسلحه-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك

الإخصائي الاجتماعي **المقر بما فيه**
التوقيع:(----------) **التوقيع:**(----------)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)
نتائج الأبحاث : (مرفق)
التشخيص النهائي : سرطان بالثدي (مبادرة)
خطة العلاج : علاج مبادرة سرطان الثدي العلاج المساعد لوقاية العظام للمرضي الذين يتلقوا علاج هرموني حمض الزيلودرونيك [14157] **قسم العلاج :** الاورام
التعليق على التشخيص : ----------

تقرير اللجنة الثلاثية المتخصصة

الاسم: د/محمد كيلاني **الوظيفة:** ---------- **التوقيع:** ----------
الاسم: د/ندى حسن **الوظيفة:** ---------- **التوقيع:** ----------
الاسم: د/نرمين رمضان **الوظيفة:** ---------- **التوقيع:** ----------

أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية.
كما تقر اللجنة بأن البيانات المدونة فى هذا النموذج صحيحة.

تعتمد صحة البيانات المرفقة
رئيس اللجنه و مدير المستشفي.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفي قبل الصرف.

أعضاء اللجنة

ا.د/ ---------- **ا.د/** ---------- **د/** ---------- **د/** ----------

قرار اللجنة

التشخيص النهائي : ----------
خطة العلاج : ----------
المستشفي : ----------**قسم :** ----------
جهة العلاج : ----------**نفقات :** ----------
ملاحظات : ----------

رئيس	مدير عام	ختم
المجلس الطبي المتخصص	مديرية الشئون الصحية	شعار الجمهورية

Name: شيماء حسن سيد فرج
National ID: 28708091402708

تقرير طبي

السيد

تحية طيبة وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بأن نوضح فيما يلي تقريرا عن حالته

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026

وبعد الكشف والفحوصات والمتابعة وجد انة

المريضة تعاني من ورم غير حميد بالثدى و تحتاج الي Zoledronic acid for bone protection during hormonal therapy

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين


جمهورية مصر العربية
بطاقة تحقيق الشخصية
شيماء
حسن سيد فرج طبطه
٤ ش سيدى الراعى -قليوب البلد
قليوب - القليوبية
١٩٨٧/٠٨/٠٩
٢٨٧ ٠٨ ٠٩ ١٤ ٠٢٧ ٠٨
JX0042433

DMS
صقاعية


٢٨٧٠٨٠٩١٤٠٢٧٠٨ ٢٠٢٣/٠٣
حاصلة على ليسانس فى الاداب والتربية
انثى مسلمة متزوجة
محمد محمد على الديب
١١ البطاقة سارية حتى ٢٠٣٠/٠٣/٠٤

مبادرة رئيس الجمهورية
لدعم صحة المرأة المصرية

05/17/2026

MDT FORM

Name:	Shimaa Hassan Sayed Farag	HRN:	5680
Age:	39 Y	ID:	28708091402708
Menopausal Sta		Phone:	01066344790
NRS SCORE:		Marital Status:	
BMI:		Family History:	
CO Morbidity:		Reason Of MDT:	

Clinical Data:

BX:

ER: 8/8 PR: 3/8 HER2: +3 KI67:

R L

Sonomammography:

Mets Workup:

History:

Recommendations:

Perimenopausal RT BREAST CANCER CT2 N1 M1 MET TO LIVER SINGLE HFL AND 2 OSSEOUS LESIONS ER8/8 PR3/8 HER2 3+ REC TAXOL AND HERCEPTIN CTS 24/11/2025 SD OF HFL BS12/2025 SD EF63%30/10 SA1.68 patient continued on herceptin taxol w30 (patient didn't receive any hormonal treatment yet
new CT CAP : stationary course

MDT Outcome & Plan

for maintaince femara zoladex/ herceptin

TNM

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric
د/ هدى عزت	د/				

Scanned with CamScanner

Name: شيماء حسن سيد فرج	
MR. : 5680	

Detailed Medical Report

السيد

تحية طيبة وبعد,,

إيماء للطلب المقدم من		**بشأن تقرير طبي للمريض**	
بقسم:		**رقم طبي:**	

ننشرف بأن نوضح فيما يلي تقريرا عن حالتها

تاريخ وساعة الدخول :		**تاريخ وساعة الخروج :**	

وبعد الكشف والفحوصات والمتابعة وجد انها

The patient was diagnosed with premenopausal right-sided breast cancer, clinical stage T2 N1 M1 with metastases to the liver (single hepatic focal lesion) and two osseous lesions. Pathology showed estrogen receptor 8/8, progesterone receptor 3/8, HER2 3+. The patient received adjuvant Taxol and Herceptin, with the last documented cycle on 24 November 2025 (stable disease of hepatic focal lesion, breast surgery in December 2025, ejection fraction 63%). On 30 October, the patient received Taxol 150 mg for weeks 19, 20, and 21, and Herceptin on day 6 of the new cycle after a one-month interval, with multidisciplinary team (MDT) review.

The patient's metastatic breast cancer progressed to bone and liver. In week 21, Taxol 150 mg was administered together with Zometa; in week 22, Taxol 150 mg plus day 7 Herceptin IV was given, with pharmacy and laboratory coordination; week 23 involved Taxol alone, followed by new echocardiogram and CT chest-abdomen-pelvis.

Pharmacy documentation for week 22 confirmed administration of Herceptin IV on day 7 and Taxol 150 mg, with an ejection fraction of 64% recorded on 9 February 2025.

Clinical pharmacy noted preparation for week 23 Taxol 150 mg for the upcoming clinic visit.

In week 24, the patient received Taxol 150 mg; week 25 included Taxol 150 mg, laboratory studies, and Herceptin on day 8; week 26 involved Taxol 150 mg with pharmacy support.

Clinical pharmacy recorded the week 25 regimen of Taxol 150 mg, labs, and Herceptin day 8, and prepared for week 26 Taxol.

Pharmacy confirmed receipt of Taxol 150 mg for week 26 and associated laboratory work on 7 March.

The patient presented for week 26 Taxol 140 mg; lower-leg edema prompted a duplex ultrasound and a CT chest-abdomen-pelvis, with pending labs, Taxol, and Herceptin pending pharmacy processing.

Clinical pharmacy noted week 27 Taxol 140 mg, day 9 Herceptin 440 mg, ejection fraction 64% (9 February 2026), and acceptable laboratory results on 28 March.

For week 28, Taxol was administered and a CT assessment was performed in March 2026; a bone scan was requested.

تحريرا في ,

التاريخ :	**21-09-2026**	**الوقت :**	

الطبيب المعالج : **dina makram**

يعتمد ,

Laboratory review on 11 April showed ALT 103 U/L and AST 59 U/L, prompting liver support measures.

For week 30, the patient received Taxol and Herceptin in a new cycle; a PET scan was pending, and calcium supplementation was considered if feasible.

Clinical pharmacy documented week 30 Taxol 140 mg, day 10 Herceptin 440 mg, ejection fraction 64% (9 February 2026), with laboratory results acceptable on 11 April.

In week 31, Taxol 140 mg and day 11 Herceptin 440 mg were administered; liver support was to be added after labs if needed, ejection fraction remained 64% (9 February 2026); CT chest-abdomen-pelvis was planned to monitor liver and bone disease, with a free echocardiogram scheduled for the next cycle; no discharge medications were required aside from calcium if available.

Pharmacy intervention noted that the patient received Herceptin on 18 April and scheduled the next Herceptin dose for the following week.

Multidisciplinary discussion recommended shifting to an aromatase inhibitor (Zoladex) pending grade 2 cellulitis and edema of the left inguinal region; a 20% dose reduction of Taxol was planned for week 32 with day 11 Herceptin 440 mg, and Taxol dose reduced to 110 mg.

MDT on 17 May outlined maintenance therapy with Femara, Zoladex, and Herceptin; CT on 17 March showed stable disease of osseous lesions in the left scapula; Taxol was to be continued until hormonal therapy approval, with a planned dose of 110 mg for week 34 and Zometa continuation.

On 15 June, the patient received Herceptin; on 27 June, a serum creatinine (CR) ejection fraction of 65% was recorded, and a non-specific left inguinal lymph node was noted on ultrasound, prompting a new venous duplex study.

On 25 July, the patient received Herceptin IV with 500 mL free saline (contrast administered), Femara, and supportive care pending CT chest-abdomen-pelvis.

CT chest-abdomen-pelvis performed on 25 July demonstrated hepatic focal lesions measuring 23 × 10 mm with a stationary course since August 2025; dynamic liver MRI and a bone scan from April 2026 confirmed stable disease; the patient remained on Herceptin 440 mg IV (cycle 15), Femara, Zoladex, and Zometa pending laboratory results for the next echocardiogram.

The most recent assessment includes a CT chest-abdomen-pelvis with comparison showing stable hepatic focal lesions (23 × 10 mm), pending liver MRI, and an echocardiogram on 31 August with ejection fraction 63%; the patient received Herceptin IV for cycle 16 and continues Femara. The patient has now completed 16 cycles of Herceptin.

NILE SCAN & LABS

مركز النيل للأشعة والتحاليل

Name : Shaimaa Hassan Sayed Farag Teita	Date : 24-04-2026 07:40
Branch : Dokki	Medical No. : 358669
Ref.Physician Prof : Goustav Roussey Egypt	Ins.Company : تعاقد مستشفيات

NILE DIAGNOSTIC CENTER

Tc-99m MDP FOLLOW UP BONE SCAN

History

39 year-old female patient with history of right breast cancer in 7/2025, received chemotherapy and referred for follow up.

Technique

- Anterior and posterior whole body scans were acquired 2 hours post injection of the tracer.

Findings

- Compared to the last bone scan dated 29/12/2025, the current scan revealed unchanged scintigraphic features in terms of size and activity of old deposits which were located at DV11 and angle of left scapula.
- No newly developed bone lesion could be detected.
- Otherwise rest of the scanned skeleton is unremarkable.

IMPRESSION

- Rather stationary course of the previously noted osseous lesions with no new osseous lesion could be depicted.
- Follow up bone scan is advised to monitor the therapeutic response.

Much Obliged
Dr. Alaa Nagy, MD
Nuclear Medicine

NILE SCAN & LABS

مركز النيل للأشعة والتحاليل

٥٦٨٠

Name : Shaimaa Hassan Sayed Farag Teita
Branch : Dokki
Ref.Physician Prof : Goustav Roussey Egypt

Date : 24-04-2026 07:40
Medical No. : 358669
Ins.Company : شبكة مستشفيات

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Much Obliged
Dr. Alaa Nagy, MD
Nuclear Medicine

NILE DIAGNOSTIC CENTER

GUSTAVE ROUSSY
INTERNATIONAL CANCER CAMPUS

Shimaa hassan SAYD

2025/2352

CT CAP +C

MULTISLICE CT SCAN OF THE CHEST, ABDOMEN AND PELVIS WITH CONTRAST

TECHNIQUE:

- Multislice axial CT images of the chest, abdomen and pelvis were obtained following IV and oral contrast administration with multiplanar reconstruction of the source images.

FINDINGS:

In the context of known right breast cancer.compared to the previous CT examination

Chest:

- Resolving consolidative patches are identified within the right upper lung region showing interval improvement compared to the previous study.
- Mild cardiomegaly is noted with enlarged cardiac silhouette.
- No definite new pulmonary nodules, consolidative patches or pleural effusions identified.
- No significant mediastinal or hilar lymphadenopathy identified.
- No pericardial collection identified.
- Patent central airways with no endobronchial lesion identified.

Abdomen:

- Mild hepatomegaly is noted.A hypoenhancing segment 4 hepatic lesion is identified measuring approximately 23 x 9 mm unchanged compared to the previous study.
- Calcular gallbladder with tiny hyperdense stone
- Normal CT appearance of the spleen, pancreas and bilateral adrenal glands.
- Both kidneys are of average size showing smooth outlines with preserved parenchymal thickness and symmetrical enhancement with no hydronephrosis.
- No significant abdominal lymphadenopathy identified.
- No ascites or free intraperitoneal fluid identified.

Pelvis:

- Bulky uterus is noted. Pelvic ultrasound is recommended for further characterization if not recently performed.
- Fecal loading noted throughout the colon consistent with constipation.
- No significant pelvic lymphadenopathy identified.
- No significant pelvic masses or collections identified.

Osseous:

- No definite lytic or sclerotic osseous lesions identified to suggest skeletal metastatic disease on the current study.

OPINION:

- No CT evidence of metastatic chest, abdominal, pelvic or skeletal disease on the current study.

Dr. Ahmed Hossam, MD

Lab Code	19232761	Sex	Female	Age	38 Y
Name	شيماء حسن سيد	Reporting Date			
Referred By	د/ الخبراء صنافير	Sample Collected Outside The lab			

(برجاء الاحتفاظ بالشرائح كسجل دائم لتشخيص الحاله)

PATHOLOGY REPORT

Specimen No.: 10529/2025
Specimen: Tru-Cut biopsy from right breast mass.

GROSS PATHOLOGY:

Three tan tissue cores measuring up to 2 cm long.

MICROSCOPIC:

Sections examined revealed breast tissue infiltrated by groups, nests, trabeculae , tubular and malformed glandular structures of malignant pleomorphic ductal epithelial with hyperchromatic nuclei and moderate degrees of anaplasia . The stroma showed moderate desmoplasia and moderate lymphocytic infiltrate.

DIAGNOSIS:

Tru-Cut biopsy from right breast mass:-
*Invasive duct carcinoma, grade 2, for immunophenotyping (ER, PR, Her-2 neu and Ki-67)

SIGNATURE
Prof Dr: Marwa Mosaad

وحدة دلالات الأورام

قسم الباثولوجيا - قصر العيني

IMMUNOHISTOPATHOLOGY

Patient Name: شيماء حسن سيد

lab.No:2025-1032

Date10/8/ 2025

Age:38 Y

Sex: Female

Markers: ER, PR, Her 2 neu and Ki67 .

Procedure:

Sections were prepared from paraffin blocks treated for (ER-clone SP1), (PR clone 1E2), (Her 2 neu-clone 4B5) and (Ki67- clone 30-9) automatically stained in Bench mark ULTRA (VENTANA) using VIEW DAB Detection Kit, Hematoxylin as counter stain

Results:

Examination of sections treated for ER reveals intense nuclear staining in most of tumor cells by chromogen.

Examination of sections treated for PR reveals weak nuclear staining <10% of tumor cells by chromogen.

Examination of sections treated for Her 2 neu revealed strong complete membranous staining >10% of tumor cells by chromogen

Examination of sections treated for Ki67 revealed positive nuclear staining in 60 % of tumor cells by chromogen.

DIAGNOSIS: RT. breast mass, TCNB:

- POSITIVE FOR ER SCORE 8/8
- POSITIVE FOR PR SCORE 3/8
- POSITIVE FOR HER2 (SCORE 3+)
- Ki67: 60%

PROF. DR. SAMIA GABAL

PROF. DR. MONA SALAH

El Mahalawy
Radiology
Center

Name:	شيماء حسن سيد
Date:	29/07/202

Non Contrast CT Examinatio[n] of the Chest Revealed:

- Few (about four in number) soft tissue nodules are seen at the right breast lower inner quadrant, the largest one about 1.5 cm. Associated right axillary multiple variable sized lymph nodes, the largest one measures about 2 cm.
- Azygos lobe and fissure (normal variation).
- Clear both lung fields with no pneumonic consolidation, pulmonary nodules or masses.
- No pleural or pericardiac sac collections.
- No pathologically enlarged hilar or mediastinal lymph nodes.
- Normal gross cardiac or vascular abnormalities.
- Intact bony thorax, no bony lesions related to the dorsal spine.
- The scanned upper abdomen revealed no gross abnormalities.

CONCLUSION:

- **Right breast lower inner quadrant soft tissue nodules with right axillary lymphadenopathy. For sono-mammographic and histo-pathological correlation.**
- **Normal C.T. study of the chest. No pulmonary metastatic deposits.**

DR. Mohamed Saber, FEBR, M.Sc

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المركز الرئيسى : قليوب البلد - شارع المدا[رس] - أمام جمعية القرآن الكريم
الفرع الثانى : قليوب البلد - شارع العاشر من رمضان (ناصية شارع العوام)

وحدة ٥٢٧٥ الأورام
قسم الباثولوجيا - قسم المناعة
IMMUNOHISTOPATHOLOGY

Patient Name: شيماء حسن سيد Lab.No:2025-1032

Date10/8/ 2025 Age:38 Sex: Female

Markers: ER, PR, Her 2 neu and Ki67 .

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- POSITIVE FOR PR SCORE 3/8
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PROF. DR. AMHA GABAL
PROF. DR. MONA SALAH

[[DOCTYPE:MRI]]
M.R.I

Ministry of Health and Population
National Hepatology & Tropical
Medicine Research Institute

TMRI

وزارة الصحة
الهيئة العامة للمستشفيات والمعاهد
التعليمية
المعهد القومى للأمراض المتوطنة

Patient Name: SHAMIA HASSAN SAYED	Patient ID: 28708091402708
Age: 39 Years, 8 Months, 26 Days	Study Date: 27/09/2026 10:51:30 AM

DYNAMIC MRI STUDY OF THE ABDOMEN

Technique of examination:

- Axial T1 (in-out phases) & T2 WIs, Coronal T2 WIs,
- Post contrast axial T1 WIs, Axial DWI & ADC

Findings:

A known case of breast cancer on chemo and hormonal therapy:

- The liver is enlarged (right hepatic span=19.8 cm), homogenous parenchyma and smooth surface.
  - **Left hepatic lobe segment IVa two adjacent subcapsular well defined focal lesions are seen measuring collectively 1.4 x 2 cm in maximum axial dimension. It displays high SI in both T1& T2WI. It elicits progressive vascular enhancement being of peripheral rim enhancement in the delayed phase of the study with element of diffusion restriction of its postero-inferior part.**
  - No dilatation of the intra hepatic-biliary radicals.
- Patent homogenously opacified portal vein and its main branches.
- Gall bladder is average distended with no signal void stones or mud.
- Normal size & MR signal of the spleen, with no focal lesions within its parenchyma.
- Normal site, size and parenchymal thickness of both kidneys with no stones or back-pressure changes.
- Normal appearance of the pancreas, aorta, IVC and both supra-renals.
- No ascites.
- No pathologically enlarged pelvi-abdominal lymph nodes.
- Normal MRI appearance of the lung bases.

Conclusion: A known case of breast cancer:

- **Hepatomegaly with left hepatic lobe focal lesion as described above likely metastatic deposit??For follow up.**

Dr. Radwa Reda

Dr. Dina Al Najjar. MD

[[DOCTYPE:BONE_SCAN]]
Bone Scan

NILE SCAN & LABS

مركز النيل للأشعة والتحاليل

Name : Shaima Hassan Said Farag	**Date : 05-10-2026 02:18**
Branch : Dokki	**Medical No. : 477191**
Ref.Physician Prof : Goustav Roussey Egypt	**Ins.Company : تعاقد مستشفيات**

NILE DIAGNOSTIC CENTER

Tc-99m MDP WHOLE-BODY BONE SCINTIGRAPHY REPORT

Clinical Indication: Follow-up and therapeutic response evaluation in a 38-year-old female with right breast cancer diagnosed in July 2025, post-surgical resection, chemotherapy, and hormonal therapy, with previously documented osteoblastic lesions at T11 and the inferior angle of the left scapula.

Procedure: Whole-body anterior and posterior planar bone scintigrams were acquired approximately 2 hours after intravenous Tc-99m MDP administration. Findings were compared with the prior studies of April 24, 2026 and December 29, 2025.

FINDINGS

- The previously reported focal radiotracer uptake at the 11th thoracic vertebra and the inferior angle of the left scapula remains stationary, with persistent low-grade intensity and no significant visual or spatial progression.
- No new focal areas of abnormally increased osteoblastic activity are identified in the remainder of the axial or appendicular skeleton, which shows homogeneous age-appropriate tracer distribution.
- Normal soft tissue background clearing with symmetrical bilateral renal excretion and expected bladder concentration.

IMPRESSION

- Stationary course of the previously documented osteoblastic lesions at T11 and the left scapular angle, with no evidence of new metastatic osseous disease, indicating stable status under the current therapeutic regimen.

Dr. Alaa Nagi, MD Nuclear Medicine

Dr. Alaa Nagy

Ministry of Health and Population
National Hepatology & Tropical
Medicine Research Institute

وزارة الصحة
الهيئة العامة للمستشفيات والمعاهد
التعليمية
المعهد القومى للأمراض المتوطنة

Patient Name: SHAMIA HASSAN SAYED	Patient ID: 28708091402708
Age: 39 Years, 8 Months, 26 Days	Study Date: 27/09/2026 10:51:30 AM

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Dr. Radwa Reda

Dr. Dina Al Najjar. MD

M.R.I
[[DOCTYPE:MRI]]